SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

# Goserelin 10.8 mg: hormone implant every 12 weeks

Locally advanced or metastatic hormone-sensitive prostate cancer · NCCP 00477 · version 3 · 84-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

**Schedule**
Goserelin 10.8 mg is placed under the skin of the front of the abdomen every 12 weeks (84 days). Your team writes the next date and the intended treatment duration.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Goserelin 10.8 mg**
GNRH AGONIST

- ☐ Hot flushes, sweating, fatigue and changes in sleep or mood
- ☐ Reduced sex drive, erection difficulties and reduced fertility
- ☐ Loss of bone strength, fracture risk, muscle loss and changes in weight
- ☐ Changes in glucose, cholesterol and cardiovascular risk
- ☐ Heart-rhythm changes; review other medicines that affect the QT interval
- ☐ Initial testosterone flare can worsen bone pain or urinary symptoms
- ☐ Injection-site bleeding; rare serious vascular injury
- ☐ Depression, allergic reactions and rare pituitary problems

Discuss treatment aim, duration, benefits and uncertainty, alternatives, sexual health and fertility, hot flushes, fatigue, mood, bone and muscle effects, metabolic/cardiovascular risks, interactions, formulation-specific risks, tests and urgent contacts. Tell the team if you take blood-thinning medicines or are very thin. These can increase bleeding risk at implantation. Severe abdominal pain, marked swelling, dizziness or collapse afterwards needs urgent help. Do not stop blood thinners yourself.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00477 v3.

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